

Clinical Evidence Dossier

Single-item Federal IDR dispute · Emergency physician critical care (CPT 99291) ·
Fictional severe septic-shock encounter.

Specimen only. Fictional case. No PHI. No real patient, provider, payer, claim, or offer. Clinovian’s scope is clinical analysis and dossier language only; eligibility, coding, offer selection, filing, portal access, fees, deadlines, legal strategy, and certification remain with the client and its designated advisers.

SECTION 01

Dispute Snapshot

QUALIFIED ITEM	Emergency physician critical care, first 30–74 minutes (CPT 99291)
DISPUTE STRUCTURE	Single-item Federal IDR dispute
CLINICAL PRESENTATION	Septic shock with acute hypoxemic respiratory failure, vasopressor initiation, and emergent source-control coordination
INITIAL PAYMENT	\$690 · fictional client-supplied field
QUALIFYING PAYMENT AMOUNT	\$720 · fictional client-supplied disclosure

INITIATING PARTY OFFER	\$2,100 · selected and approved by client
NON- INITIATING PARTY OFFER	\$720 · shown separately even where numerically identical to QPA
CLINOVIAN SCOPE	Clinical evidence synthesis, record mapping, and narrative QA only

SECTION 02

Clinical Evidence Map

CLINICAL FACT	RECORD SUPPORT	WHY IT MATTERS
Shock at presentation	BP 78/46 after initial fluid bolus; lactate 5.2 mmol/L; mottled extremities	Establishes immediate life-threatening instability and high-risk medical decision-making.
Vasopressor management	Norepinephrine initiated and titrated through central access with continuous reassessment	Supports active critical-care management rather than routine emergency evaluation.
Respiratory failure	SpO ₂ 84% on room air; escalating oxygen support; arterial blood gas and airway-readiness documentation	Demonstrates concurrent organ dysfunction and resource intensity.

CLINICAL FACT	RECORD SUPPORT	WHY IT MATTERS
Source-control coordination	Urgent surgical consultation, broad-spectrum antibiotics, serial reassessment, transfer to operating room	Shows time-sensitive multidisciplinary management with high risk of deterioration.
Critical-care time	58 minutes of non-duplicative physician critical-care time documented	Supports the clinical intensity of the disputed service, subject to client coding validation.

SECTION 03

Five Enumerated Additional Circumstances

1 — Training, experience, quality and outcomes. The treating physician's emergency-medicine certification, critical-care experience, and client-supplied quality data may be relevant if verified and connected to the disputed service. Generic credential statements are excluded.

2 — Market share of the parties. Market-share evidence is outside the medical record. Clinovian flags it for client or counsel validation and does not infer it from the clinical file.

3 — Patient acuity or complexity of furnishing the service. Strongly supported. Shock, respiratory failure, vasopressor titration, serial reassessment, and emergent source-control coordination materially distinguish the encounter from a routine high-acuity ED visit.

4 — Teaching status, case mix and scope of services. Potentially relevant if the client verifies the facility's status and explains how its capabilities were material to this encounter. The dossier does not assume that facility status alone changes value.

5 — Good-faith contracting efforts and prior contracted rates. Client-supplied contracting evidence may be included where applicable. Clinovian does not convert contracting history into a clinical conclusion.

SECTION 04

QPA and Offer Separation

WHAT THE DOSSIER DOES NOT SAY

The QPA is not labeled as the payer's offer, and the initial payment is not treated as interchangeable with either figure. The dossier does not claim that the QPA is invalid or incapable of reflecting any complexity.

WHAT THE CLINICAL RECORD SUPPORTS

The case-specific severity, risk, and resource intensity documented here may not be fully captured by the QPA or initial payment. The client may use this evidence, together with other credible permissible information, when supporting its independently selected offer.

SECTION 05

Missing-Evidence and Duplication Check

- Confirm that the critical-care time is non-duplicative and supports the submitted code.

- Obtain the plan's complete QPA disclosure and keep the QPA distinct from both party offers.
- Validate any market-share, facility, quality-outcome, and contracting evidence outside the medical record.
- Remove clinical facts already incorporated into the QPA or repeated across multiple circumstances without additional relevance.
- Exclude billed-charge, usual-and-customary, and public-payer-rate arguments from the clinical dossier.

SECTION 06

Physician Clinical Synthesis

Clinical conclusion for client review. The record supports a materially complex critical-care encounter involving hemodynamic shock, respiratory failure, vasopressor management, serial reassessment, and urgent source-control coordination. Those case-specific facts provide a credible clinical basis for distinguishing the service from a routine emergency encounter. The client retains responsibility for deciding how that evidence relates to its offer and for all procedural and legal submissions.

SINGLE-ITEM DISPUTE

CLINICAL EVIDENCE MAP

FIVE CIRCUMSTANCES

QPA SEPARATED

NO PORTAL ACCESS

This specimen demonstrates the clinical evidence layer only. A live engagement begins with a de-identified suitability screen, written fixed scope, BAA where required, and secure transfer instructions. Clinovian does not determine IDR eligibility, validate coding, select offers, access the portal, pay process fees, or provide legal representation.

[NSA/IDR service detail](#) · [Request a de-identified suitability screen](#) · [Privacy](#) · [Terms](#)